

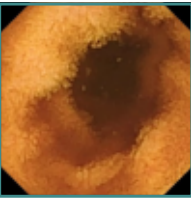
Lane A: passive-video forecasting

validated across three orifices



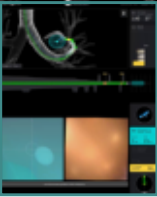
CholecT50 (laparoscopy)

last observed frame



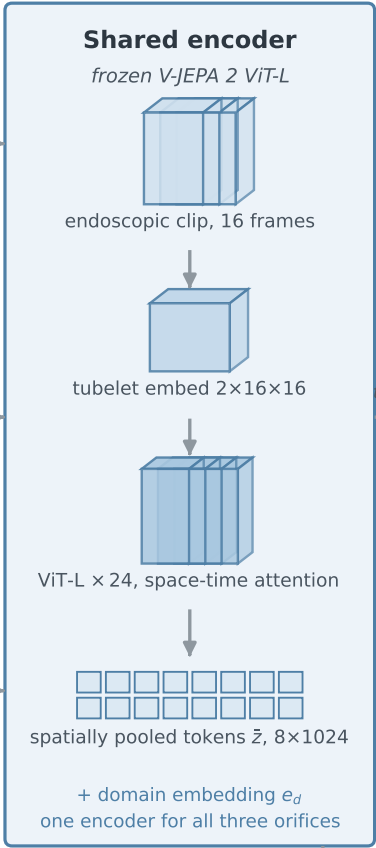
Kvasir-Capsule (GI)

last observed frame



ION (bronchoscopy)

navigation-console capture



Temporal predictor heads

two temporal scales, encoder space only

L1: pooled causal residual

GPT-style causal transformer
horizon 4 tubelets
frozen-teacher targets

pooled state

L2: pooled coarse residual

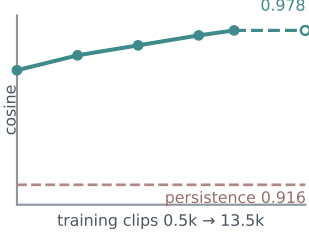
temporally pooled tokens
implemented coarse extension
not headline-evaluated

cosine target; no pixel decoder

forecast $\hat{z}$

Capability 1: forecasting

predict future encoder states

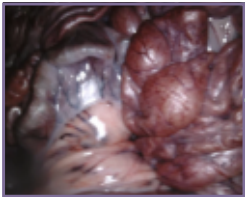


cosine 0.978 vs 0.916 persistence
shared 750-clip validation protocol
monotone gain, plateau at 13,552 clips
video-level split, held-out validation

same frozen tokens

Lane B: physical grounding

audited on pose-gated data only



SCARED (laparoscopy)

frame + calibrated pose

Tubelet-aligned SE(3) actions



$u_t = \log(T_t^{-1}T_{t+1})$, camera frame
one twist per 2-frame tubelet
depth gives near-wall risk labels

twist u

SE(3) latent dynamics



block-causal action path
probabilistic ensemble, μ , Σ
risk head, counterfactual loss

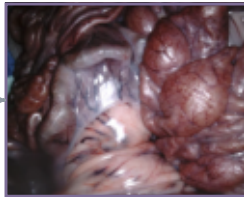
$\hat{z}$, risk

SE(3)-conditioned evaluation



deranged batch action win 87.0 % ($n = 958$)
oracle-goal proxy win 60.0 % (200 windows)
risk AUC 0.523 < 0.75; filter inactive

retrieval proxy



SCARED (laparoscopy)

SE(3)-conditioned retrieval

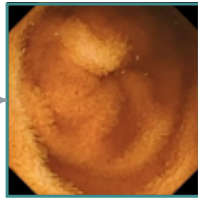
loss-of-view warning



CholecT50 (laparoscopy)

predicted-latent retrieval

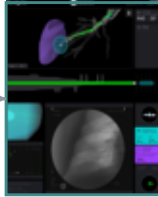
camera-handling training



Kvasir-Capsule (GI)

predicted-latent retrieval

cross-orifice representation



ION (bronchoscopy)

predicted-latent retrieval

Correctness audits applied to both lanes

Input-sensitivity tests
history / action / domain

C3VD depth-warp diagnostic
convention not validated

Action negatives
batch shuffle / fixed bank

Grouped CV, logged test contacts
audit-selected result